

1. Administrative & Study Identification

Full Study Title: A Randomized, Open-Label, Phase 3 Trial of Tisotumab Vedotin vs Investigator's Choice Chemotherapy in Second- or Third-Line Recurrent or Metastatic Cervical Cancer
Short Title/Acronym: Tisotumab Vedotin vs Chemotherapy in Recurrent or Metastatic Cervical Cancer / innovaTV 301 / ENGOT-cx12 / GOG-3057 **Protocol Number:** SGNTV-003 / C5721002 **NCT Number:** NCT04697628 **Sponsor/Company Name:** Seagen, a wholly owned subsidiary of Pfizer (in collaboration with Genmab, ENGOT, and GOG) **Investigational Product:** Tisotumab vedotin-tftv (Tivdak) **Phase of Development:** Phase 3 **Trial Status:** Active, not recruiting **Medical Monitor:** Pfizer/Seagen Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate whether tisotumab vedotin demonstrates a statistically significant improvement in Overall Survival (OS) compared to investigator's choice chemotherapy in patients with previously treated recurrent or metastatic cervical cancer. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), confirmed Objective Response Rate (ORR) by investigator, time to response (TTR), duration of response (DOR), safety, and health-related quality of life outcomes.

2.2 Background & Rationale Recurrent or metastatic cervical cancer (r/mCC) is a devastating, mostly incurable disease with a critical need for survival-extending treatments after progression on first-line systemic therapies. Tisotumab vedotin is a tissue factor-directed antibody-drug conjugate (ADC) that previously received accelerated approval for this indication. The innovaTV 301 study serves as the confirmatory phase 3 trial designed to demonstrate that tisotumab vedotin yields a statistically significant and clinically meaningful overall survival benefit over standard-of-care single-agent chemotherapy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Investigator's choice of single-agent chemotherapy: topotecan, vinorelbine, gemcitabine, irinotecan, or pemetrexed) **Blinding:** Open-label **Randomization:** 1:1

3.2 Planned Enrollment & Duration Target Enrollment (\$N\$): 502 participants **Number of Sites:** Global multicenter trial (including sites in the US, EU, Japan, Latin America, Taiwan, Singapore, and South Korea) **Estimated Study Start:** 22-Feb-2021 **Estimated Primary Completion:** 24-Jul-2023

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with recurrent or metastatic cervical cancer (squamous cell, adenocarcinoma, or adenosquamous histology). 2. Disease progression during or after treatment with a standard of care systemic chemotherapy doublet $\pm$ bevacizumab and/or an anti-PD-(L)1 agent (if eligible). 3. Received 1 or 2 prior systemic therapy regimens for r/mCC. 4. Measurable disease according to RECIST v1.1 as assessed by the investigator. 5. ECOG performance status of 0 or 1 and life expectancy of ≥ 3 months.

4.2 Exclusion Criteria 1. Active ocular surface disease, history of cicatricial conjunctivitis, or inflammatory conditions predisposing to cicatrizing conjunctivitis (e.g., ocular Stevens-Johnson syndrome). 2. Clinically significant bleeding issues or risks (e.g., known coagulation defects, ongoing major bleeding). 3. History of intracerebral arteriovenous malformation, cerebral aneurysm, or stroke. 4. Peripheral neuropathy $\geq$ Grade 2. 5. Any prior treatment with monomethyl auristatin E (MMAE)-containing drugs.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Experimental Arm:** Tisotumab vedotin 2.0 mg/kg (maximum of 200 mg for patients ≥ 100 kg) every 3 weeks (Q3W). * **Active Comparator Arm:** Investigator's choice of Topotecan, Vinorelbine, Gemcitabine (ATC Code: L01BC05; Trade Names: Gemzar, Infugem), Irinotecan, or Pemetrexed according to standard dosing schedules. **Route of Administration:** Intravenous (IV) infusion (over 30 minutes for TV). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, withdrawal of consent, or study closure.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care to manage adverse events (e.g., antiemetics, proactive eye care). **Prohibited:** Concurrent MMAE-containing drugs or participation in another therapeutic clinical trial.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Baseline Tumor Assessment, Ocular Exams

Baseline | Day 0 | Randomization (1:1), First IV Infusion, AE Education | | **Interim** | Every 3 Weeks | Treatment administration, Safety & AE Labs | | **Tumor Evaluation** | Weeks 6-30 | Disease assessment via RECIST v1.1 every 6 weeks, and every 12 weeks thereafter | | **End of Study** | Variable | Treatment Discontinuation, Final Safety & Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS), defined as the time from randomization to death due to any cause.

Measurement Method: Clinical tracking and mortality documentation.

7.2 Secondary & Exploratory Endpoints * Progression-Free Survival (PFS) by investigator assessment * Confirmed Objective Response Rate (ORR) by investigator * Time-to-Response (TTR) as Assessed by the Investigator * Duration of Response (DOR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard vital and lab monitoring with special focus on ocular adverse events (e.g., conjunctival reactions), peripheral neuropathy, and bleeding incidents (epistaxis). **Serious Adverse Events (SAEs):** Reported within 24 hours to the sponsor and regulatory authorities. **Data Monitoring Committee (DMC):** An Independent Data Monitoring Committee (IDMC) oversaw the study to evaluate interim analyses for OS and safety thresholds.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for efficacy consisting of 502 randomized participants. Safety population includes all patients who received at least one dose of the study drug. **Sample Size Justification:** Powered to detect a statistically significant reduction in the risk of death (OS benefit). At the interim analysis, the threshold for OS improvement was successfully crossed. **Handling of Missing Data:** Standard right-censoring methods for time-to-event survival analyses (e.g., Kaplan-Meier estimates).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring of eCRFs, specifically tracking adherence to the required proactive eye

care protocol. **Audit Trail:** Secure database locking, query resolution via data clarification forms, and independent radiological/clinical reviews.

11. Study Identifiers & Governance

Primary ID: SGNTV-003 **Posting ID:** 417445594725507938 **Registry Number:** NCT04697628 **Sponsor/Lead Organization:** Seagen Inc. (a Pfizer company) **Study Title:** innovaTV 301 **Official Regulatory Title:** A Randomized, Open-Label, Phase 3 Trial of Tisotumab Vedotin vs Investigator's Choice Chemotherapy in Second- or Third-Line Recurrent or Metastatic Cervical Cancer

12. Clinical Framework (Oncology Specific)

Disease Area: Cervical Cancer **Primary Condition:** Recurrent or Metastatic Cervical Cancer (UMLS: Metastatic Cervical Carcinoma) **Diagnosis Threshold:** Progression during or after standard chemotherapy doublet **Medical Methodology:** Tissue factor-directed Antibody-Drug Conjugate (ADC) vs. Chemotherapy **Optimization Target:** Prolonged Overall Survival (OS) and Progression-Free Survival (PFS)

13. Study Procedures & Methodology

Management Pathway: Intravenous antineoplastic therapy **Education Protocol (HCP):** Identification and management protocols for ocular surface toxicity, bleeding diathesis, and peripheral neuropathy. **Education Protocol (Patient):** Mandatory adherence to proactive eye care (e.g., lubricating eye drops, vasoconstrictor drops, and cooling eye masks during infusions). **Quality Audit Mechanism:** RECIST v1.1 radiographic assessment tracking and IDMC safety reviews.

14. Observation & Follow-Up Schedule

Total Duration: Continued until disease progression, unacceptable toxicity, or study closure. **Onsite Visit Cadence:** Every 3 weeks (Q3W) for treatment cycles. **Remote Monitoring Frequency:** Routine inter-cycle safety checks for severe AEs. **Checklist Review Interval:** Radiographic tumor assessment every 6 weeks for the first 30 weeks, then every 12 weeks thereafter.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				

Clinical Operations Lead | [Name] | ____ | [DD-MMM-YYYY] | | Lead
Statistician | [Name] | _____ | [DD-MMM-YYYY] |